

East Bay Family Medicine

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Patient: David Park
Date of Birth: 09/24/1986
Patient ID: 7782041
Date of Encounter: October 28, 2024
Encounter Type: Office Visit, Follow-up
Provider: Marcus Hayes, MD

SUBJECTIVE

Mr. Park returns for follow-up of right L5 radiculopathy. He underwent right L4-L5 transforaminal epidural steroid injection on 09/09/2024 with Dr. Liu at East Bay Pain Management.

He reports approximately 70% improvement in his radicular symptoms since the injection, with the most significant improvement occurring in the second and third weeks post-procedure. The "electric" sensation down his right posterior thigh has essentially resolved, and the lateral calf discomfort is now intermittent and mild rather than constant. His back pain at rest is minimal — perhaps a 1-2/10 on most days. He still notices increased symptoms with prolonged sitting beyond 90 minutes, but this is much improved from baseline.

He has resumed running with a walk-run progression as guided by his physical therapist. He completed a 2-mile run last weekend without significant symptoms. He has hiked twice with his partner over the past month, including one hike of approximately 4 miles, with mild post-activity soreness but no flare of radicular symptoms.

He has completed his prescribed course of physical therapy and is doing the home exercise program 4-5 days per week. He has discontinued routine NSAID use and takes naproxen only occasionally (1-2 times per week) for residual stiffness.

His employer approved a sit-stand desk and an external ergonomic keyboard, which he reports has helped substantially. He alternates between sitting and standing approximately every 30-45 minutes during the workday.

OBJECTIVE

Vitals: BP 118/76, HR 68, weight 174 lb (down 4 lb from June). General appearance: well, no acute distress. Gait normal. Lumbar spine examination: no longer significant tenderness over the right paraspinal region. Forward flexion is essentially full and pain-free. Right straight-leg raise is now negative through 80 degrees. Strength is 5/5 throughout the lower extremities, including hip abductors and EHL bilaterally. Reflexes are 2+ and symmetric, including Achilles bilaterally — the previously noted right Achilles asymmetry has resolved on today's exam. Sensation intact.

ASSESSMENT

1. Right L5 radiculopathy 2/2 right L4-L5 disc protrusion — much improved. Patient has had a robust response to a structured care plan: 6 weeks of PT, transforaminal epidural steroid injection, ergonomic modification, and progressive return to activity. Examination findings have normalized.
2. Sustained lifestyle modifications including weight loss of approximately 4 lb, ergonomic improvements, and consistent home exercise program.

PLAN

1. No further interventional procedures planned at this time. Will plan to defer additional injections unless symptoms recur or escalate.
2. Continue home exercise program. Discussed the importance of long-term consistency given the underlying disc anatomy that has not changed structurally despite the symptomatic improvement.
3. Discussed the natural history: most patients with this anatomy do well long-term with maintenance of core stability and ergonomic discipline. Recurrence is possible, particularly with re-injury, but is not expected with adherence to the current program.
4. Patient will follow up as needed. Routine return visit not scheduled. He understands to return promptly for any new or progressive neurologic symptoms, particularly any motor weakness or bowel/bladder changes (counseled to seek emergency evaluation for the latter).
5. I will not pursue additional imaging at this time given the patient's clinical improvement.

Total time spent: 20 minutes, of which the majority was discussion of long-term maintenance strategy and natural history.

— *Marcus Hayes, MD*
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